

September 15, 2016

Docket FDA-2015-D-3719

Please consider the following additional comments, following the public hearing on the Draft Guidances Relating to the Regulation of HCT/Ps on September 12 and 13, 2016

1. With respect to the issue of whether the concept of “main function” to identify tissue as structural or non-structural is new, or is a concept FDA has always applied—

It is sometimes asserted that FDA has applied the concept of main function to minimal manipulation determinations for many years. This assertion is sometimes supported by a reference to this statement in the May 14, 1998 preamble to the Establishment Registration & Listing proposed rule: “FDA recognizes that some products may have both systemic and structural effects but intends that a product’s *primary effect* be determinative.” Emphasis added. See the Federal Register of May 14, 1998, 63 FR 26744, 26749.

It must be noted that FDA made the above statement in the context of a discussion of the fourth criteria for eligibility for marketing solely under Section 361 of the Public Health Service Act, that is, whether or not the HCT/P has a systemic effect. See 21 CFR 1271.10(a)(4)(i). This statement was not made in the context of a discussion of minimal manipulation, (21 CFR 1271.10(a)(1), nor was it made in the context of a discussion of how to determine whether tissue should be considered structural or non-structural.

The cited FDA statement does not support the assertion that FDA has long applied the concept of main function to the identification of tissue as structural or non-structural.

2. The Draft Guidance on HCT/Ps from Adipose Tissue states, in Example B-3, page 5:

The basic function of breast tissue is to produce milk (lactation) after childbirth. Because this is not a basic function of adipose tissue, using HCT/Ps from adipose tissues for breast augmentation would generally be considered non-homologous use.

Several speakers at the public hearing noted that breast tissue has functions in addition to lactation.

In addition, it should be noted that FDA’s statement misapplies the homologous use regulations. According to 21 CFR 1271.3(c), homologous use means the:

...repair, reconstruction, or supplementation of a recipient’s cells or tissues with an HCT/P that performs the same basic function or functions in the recipient as in the donor.

Under the regulations, the correct analysis is to compare the function of the donated tissue in the donor, and the function the donated tissue is expected to perform in the recipient.

The Draft Guidance on HCT/Ps from Adipose Tissue indicates that in the donor, adipose tissue stores energy, insulates the body, and provides cushioning and support for subcutaneous tissues and internal organs. (Draft Guidance, page 1.)

Therefore, in determining whether breast augmentation is a homologous use of adipose tissue, the correct question is “Is transplanted adipose tissue expected to store energy, insulate the body, or provide cushioning and support for subcutaneous tissues and internal organs?”

The correct question is not “Does adipose tissue in the donor have the same basic function as breast tissue in the recipient?”

These two questions may have different answers. It is important to ask the correct question.

Thank you for your consideration of these additional comments.

Respectfully,

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